

as approximately 700 sq. ft. 2-BHK homes. The project value is estimated around ₹300 crore, with prices starting from approximately ₹60 lakh,” says Hiranandani.

He plans to leverage current group capabilities across health-care, hospitality, education, retail, and entertainment, giving them an ecosystem capable of supporting the development of integrated senior communities.

Luxury players like DLF have also announced their entry into the sector with The Aureva Senior Living in Sector 63A, Gurugram, with units costing over ₹10-11 crore each.

A March 2025 report by CRE-DAI and KPMG identifies freehold sale, pure rental and leasehold as the three principal development structures.

Narahari says over 60-70% senior buyers opt for outright ownership. Pure rental, leasehold and care-led, assisted-living models, where accommodation and services are bundled into a monthly payment, account for the remaining 30%. Increasingly, as more seniors seem reluctant to lock up their retirement corpus in another home, the industry believes lease will become more significant.

AND THEN COMES CARE

However, senior living is about more than buying an apartment. It also brings a range of services and care into the equation.

“Senior living and senior care are not a single format,” explains Raja Gopal of ASLI, who is also Co-Founder, Director & CEO, Lifebridge Group, promoting Serene Communities by Columbia Pacific and KITES geriatric care. “Seniors will require different levels of support at different stages of ageing—from independent living and preventive wellness to home care, assisted living, rehabilitation,



“People don’t necessarily want somebody to ‘look after’ them; they want people to have dinner with, travel with, exercise with and pursue interests with”

ADARSH NARAHARI

FOUNDER & MD,
PRIMUS SENIOR LIVING &
MARZI BY PRIMUS



“Today’s seniors are more financially independent, more mobile, and increasingly willing to invest in quality of life. Their expectations are closer to those of a global consumer”

**NIRANJAN
HIRANANDANI,**

CO-FOUNDER AND
MANAGING DIRECTOR, THE
HIRANANDANI GROUP

dementia care and palliative care.” These require strong linkages with hospitals, diagnostics, pharmacies, rehabilitation providers, technology, emergency response systems, nutrition, and wellness services.

Gupta says more than 25-30% of the population in Ashiana utilises nursing, outpatient facilities, physiotherapy, visiting doctors, dentists, and other healthcare services. Just 10% of residents use assisted living or caregiving support.

In Manasum’s format, the senior buyer pays the real estate cost and senior care charges, which includes common area maintenance (CAM), senior amenities maintenance charges (SAM), and one-time senior set up charges.

For independent senior living services, Narahari estimates that a reasonable broad industry range today would be around ₹15,000-₹30,000 per month per household for the core service layer, with premium projects going to ₹25,000-₹50,000-plus, depending on what is included.

Assisted living is a different category because manpower intensity changes dramatically. Current metro-market examples are often ₹40,000-60,000 a month and can go substantially higher as nursing, dementia or one-on-one care requirements increase.

MOVING INTO SMALL-TOWN INDIA

The senior living model is also moving beyond India’s metro cities and into smaller cities and towns.

“Over the next three-four years, as awareness and acceptance of senior living projects continue to grow and expand beyond Tier I cities, emerging Tier II/III cities will see their share rising to 30-40%, broadening the segment’s geographic footprint across the country,” says Vimal Nadar, National